
CONSULT FOR AFRICA

Arabella Women's Health

Diagnostic Audit: Information and Data Request

Prepared by Dr Debo Odulana, Founding Partner, Consult for Africa

For Dr Chito Nwana and the Arabella Women's Health leadership

Revised 9 October 2026 / Audit under way, written diagnostic by the end of November

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A note before the list

Dr Chito, thank you, and thank Tolu and the team for me. The sales record, the price list, the staff list, the HMO list, the pharmacy register, the patient list and the company certificate all came in within a day of asking, and thirteen of your staff have already answered the survey. That is quicker than we usually see.

From them we now know what Arabella takes in a month and where it comes from, and exactly what we cannot see yet. The biggest gap is cost. Nobody, including us, can say which services make money until we know what each one costs to run, and that is why it sits near the top of the list below.

I have also rewritten the request around what you told us about Tabitha. It is your practice changing its name, not one business taking over another, so the questions about the change are now only about what still needs moving into Arabella's name. I have also regrouped the list so that each question is asked once.

Nothing in this list is a test. Where a record does not exist, tell us so, because that absence is itself useful. Your numbers, records and your staff's answers stay with Consult for Africa and are not shared with anyone unless you tell us in writing.

Warm regards,

Dr Debo Odulana

How to send it

Everything goes through the same private page: consultforafrica.com/ArabellaProject. Choose the section a file answers, or "One of the eight we need next", and send things as you find them. Exports from a system beat typed summaries, photographs of paper registers are fine, and where something is easier to explain in person, write "discuss" against it and we will cover it on site.

Clinical case notes that name a patient are the one thing we would rather see on site than receive, on your own machine. The patient list you sent is held under the confidentiality above, and used only for this work.

What has come in

What	Status	What it told us, or what is still missing
Company certificate	In	RC 1802696, Arabella Women's Health & Wellness Ltd, incorporated 7 June 2021, renamed 9 September 2024
Sales record, January to September 2026	In	Every payment by service and channel. Still to come: October to December 2025, so we have twelve months, and the summary sheet for August and September
Price list	Partly	The laparoscopic procedures are marked TBD, there are no fertility prices, and the Platinum package price needs correcting
HMO list	Partly	Seven insurers named. Tariffs, contracts and what each owes are still to come
Staff list	In	Roles, days and reporting lines. Pay is asked for in section E
Pharmacy stock register	Partly	Quantities and expiry dates. Unit costs are still to come
Patient list	In	4,379 records



What	Status	What it told us, or what is still missing
Staff survey	13 answers	Still open to anyone who has not answered
Leadership survey	2 answers	Thank you both

The eight we need next

These are the gaps that stop us answering the audit's main questions. If time is short, send these first.

#	What we need	Why it is next
1	Bank statements for every account the practice is paid into, last 12 months. The sales record shows First Bank, GTB and Access	The only way to confirm that what the record says came in actually reached the bank
2	Monthly costs for the last 12 months: salaries by person, rent, diesel and power, drugs and consumables bought, laboratory tests sent out, and everything else paid regularly	Without costs nobody can say which services make money. This is the most important gap
3	For each HMO and company account: the tariff, the contract, and what it owes today with the oldest unpaid claim	Insurers paid under 2% of what came in this year, although nearly 500 registered patients are insured or covered by an employer
4	The fertility service: who does the laboratory work and the embryology, how many cycles were started this year, and what a cycle is charged	Fertility brought in N6m this year in a single payment, and it is not on the price list or the staff list
5	The laboratory: which tests are done on site and which are sent out, to whom, and what each costs Arabella	It is the largest line in the sales record, at 30% of everything taken
6	Maternity this year: women booked, babies delivered at Arabella, booked women who delivered elsewhere and why, and caesarean sections	Maternity is where the brand is pointed, and the sales record shows two to four packages in nine months
7	The facility registration certificate, and the name it is in	Whether the registration covers what is delivered, and whether it moves to Arabella's name
8	Prices for the laparoscopic procedures now marked TBD	Surgery is one of the lines the growth plan depends on, and it cannot be quoted without a price

A. The company and the change of name

Already in: the certificate of incorporation.

- The CAC status report for Arabella, with the current directors and shareholders, and whether any shareholder is not Nigerian
- Tabitha Medical Centre's registration: a company or a business name, its number, and the plan and timing for closing it
- What still sits in Tabitha's name and needs moving: bank accounts, HMO contracts, the facility registration, the premises lease, signage and stationery, the website and domain, the social media



accounts and the phone lines. A short list is enough

- What has already been told to HMOs, staff, suppliers and patients about the change, and in what form
- Any tax, PAYE or pension position left open in Tabitha's name
- How decisions are made: who meets, how often, who signs off spending above what amount, and the bank mandate
- Any loan, overdraft, equipment finance, or money you have put in personally, with its terms
- Any other clinician with a share of the revenue or of the business

B. Licences, insurance and compliance

- The facility registration with the FCT health authority: the category, the name it is in, the expiry date and any conditions, plus any inspection report
- The medical director of record for the premises
- MDCN practising licences for every doctor who works here, including visiting consultants and the anaesthetist, and Nursing and Midwifery Council licences for every nurse and midwife
- For the fertility service: the responsible clinician, the embryologist's qualification and registration, and any accreditation held
- Pharmacy premises licence and superintendent pharmacist; the controlled drugs register and the last reconciliation
- Medical Laboratory Science Council registration for laboratory work done on site
- Radiation licence and dosimetry, if any X-ray equipment is held
- Professional indemnity for every clinician and for the facility; public liability, fire and equipment insurance
- Clinical waste contract and consignment notes; fire safety certificate
- Registration with the Nigeria Data Protection Commission, and who the data protection officer is
- Any current or threatened claim, complaint to a regulator, or litigation

C. Money in

Already in: the sales record, January to September 2026.

- The sales record for October to December 2025, so that we have a full twelve months
- The summary sheet for August and September 2026
- Bank statements, as in item 1 above, and whether any business money has been paid into a personal or staff account
- The two largest payments this year, March's N6.5m for fertility and September's N2m procedure: what each was for, and whether either was paid in instalments
- Registrations: the patient list shows 145 new registrations this year and the sales record 83 registration fees. Is the fee waived for some patients, and for whom?
- Cash handling: who takes money at the front desk, how it is receipted, when it is banked and who checks it against the day
- Any money owed to Arabella by patients, and how it is chased

D. Prices, packages and who pays

Already in: the price list and the list of HMOs.



- Prices for the procedures marked TBD and for fertility treatment, including what a cycle includes, how drugs are charged and what a frozen transfer costs
- What each maternity package includes and, precisely, what it excludes: what happens on a caesarean, a longer stay, a baby needing admission, a transfer out, or twins
- How a package is paid, and what happens when a patient stops paying part way through
- Who may give a discount, and the discounts given this year
- For each HMO: the tariff, the contract and its date, the contact person, whether it is active or lapsed, and whether it is in Arabella's or Tabitha's name
- HMO and company receivables: what each owes and for how long, claims rejected this year with the reasons, and who chases them
- The company accounts in the patient list, including Central Bank of Nigeria, ECOWAS, Shoprite, Prowess Energy and the International Community School: the terms of each, and whether they are still active
- How an authorisation code is obtained, and what the front desk does when it is slow
- The last five quotes that did not turn into treatment, and what is known about why

E. Money out

- Monthly costs for the last 12 months, as in item 2 above, in whatever form they are kept
- Payroll by person and month, including locums and visiting consultants, and any commission or bonus
- Purchases of drugs and consumables with supplier and price, and any supplier credit or money owed
- What laboratory tests sent out cost, by month
- The café: what it takes and what it costs to run
- Marketing spend in the last 12 months, by channel, including the social media and podcast teams
- Rent and the lease terms; diesel and power; equipment and its finance
- Any management accounts, profit and loss or balance sheet, and the accountant's details

F. How each service runs

Maternity. Women booked and babies delivered this year, booked women who delivered elsewhere and why, caesarean rate, the route for transferring a mother or baby who needs intensive care and the last three times it was used, how quickly blood reaches theatre in a haemorrhage, and anaesthetic cover out of hours.

Fertility. Cycles started, cancelled before transfer, transfers and pregnancies this year by age band; who does the embryology; consent forms for storing eggs and embryos; how the storage tanks are monitored and what happens in a power cut.

Gynaecology surgery. Procedures done this year, cancellations on the day and why, and who provides anaesthesia and theatre staff.

Laboratory. Tests done on site and tests sent out, which laboratory receives them, turnaround time, and the price Arabella pays for each.

Pharmacy. The stock register with a unit cost for each item, purchases this year, stockouts in the last three months, expired stock written off, and how the cold chain is kept.

Wellness. What is offered, by whom, and how it is paid.

G. Clinical safety



- Written protocols for postpartum haemorrhage, pre-eclampsia and eclampsia, shoulder dystocia, newborn resuscitation and sepsis, and when each was last practised
- The resuscitation trolley check, and life support training with expiry dates
- Consent forms for delivery, surgery and fertility treatment
- Incident reports this year and what changed as a result; any maternal or newborn death, stillbirth or unexpected transfer in the last two years, and the review that followed
- Sterilisation: autoclave validation and load records
- Infection prevention: the last audit, cleaning schedules and sign-off
- Complaints received in any form, including on social media, and how each was answered

H. Patients and where they come from

Already in: the patient list.

- What patients have been told about how their details are used, and any consent or privacy notice they signed
- Whether anyone has asked not to be contacted, and how it is recorded
- How a follow-up appointment is booked before a patient leaves, and who checks that she came back
- The patient journey as it runs: who answers the phone and WhatsApp, how a booking is confirmed, waiting times, and who follows up
- Opening hours, including weekends
- Where new patients come from: which doctors, clinics, companies or people refer, how many each sent this year, and anyone who used to refer and has stopped
- Enquiries a month by channel, and how many become a booking
- HERcast: listeners by platform, and any sponsorship
- Any outreach already sent to the patient list, when, and what came back

I. People

Already in: the staff list.

- Contracts and job descriptions, and the staff handbook or policies if any
- The rota for the last three months, and how nights, weekends and holidays are covered
- Who covers the front desk, and who covers when that person is away
- Staff who left in the last two years, from which role, and why
- Training records and any appraisal or performance conversations
- How tasks are given out and checked: who decides what needs doing, how the person doing it knows, and how anyone knows it was done

J. Systems and records

- Every system in use, what it is for, who supplies it and what it costs, including the one the patient list came from
- Who administers each, who holds the passwords, and whether any login is shared
- What is backed up, where, and when a restore was last tested
- Who can see patient records, and whether access can be traced to a person



- Any record kept only on a personal phone, email account or drive
- The website: who built it, who holds the domain, and who can publish
- Google Business Profile, Google Ads and Meta Business accounts: who owns them and who has admin

How the audit runs from here

Stage	What happens	What we need from the team
Records	Revenue reconstructed and traced to the bank. Costs put against each service	The eight above, then the rest as it is found
Conversations	Short, confidential conversations with staff. The surveys stay open	Twenty minutes per person, around clinical work
On site	Patient journey traced on live cases. Pharmacy and stock counted against costs	Access to the day as it actually runs
Findings	Findings tested with you before anything is written	An hour of your time
By the end of November	Written diagnostic: what Arabella earns, what each service makes, where it leaks, and what to fix in what order. Technology and records report with a prioritised roadmap	A session with you to walk through it

One last thing

If something on this list does not exist, the honest answer helps us more than a reconstruction. If something exists but is embarrassing, send it anyway. The practices that improved fastest were always the ones that showed us the real thing at the start.

Anything at all, call me directly.

Dr Debo Odulana

Founding Partner, Consult for Africa